

NHS Devon Integrated Care Board Service Specification

Service Specification No.	
Service	Patient Pathway Optimisation LES (Devon Wide)
Commissioner Lead	Paul Green, Director for Primary Care
Provider Lead	
Period	1 st April 2026 to 31 st March 2027 (3 months' notice will be required to terminate this contract by either party)
Date of Review	By March 2027

1. Population Needs

1.1 National/local context and evidence base

GP practices, dental practices, optometric practices and community pharmacies are currently commissioned to provide essential, additional and specified enhanced levels of service, principally through national contracting routes.

In addition, commissioning entities including Clinical Commissioning Groups may commission additional services from those providers. As with all such additional (none core contract) services, GP practices elect to provide them without there being any requirement to do so. If practices do not elect to provide, or subsequently serve notice on provision, it is the responsibility of the commissioner to ensure alternative pathways and delivery models exist.

1.2 Local Context

This specification seeks to ensure provision of a range of services which NHS Devon wishes to commission from General Practice in the Devon area to optimise local patient pathways. This being in line with its stated commissioning intentions to, where appropriate, provide care closer to patients and within community settings.

2. Outcomes

2.1 NHS Outcomes Framework Domains & Indicators

Domain 1	<u>Preventing people from dying prematurely</u> Reducing deaths in babies and young children	
Domain 2	<u>Enhancing quality of life for people with long-term conditions</u> <u>Health related quality of life for people with long term conditions</u> <u>Ensuring people feel supported to manage their condition</u> <u>Improving functional ability in people with long term</u>	✓

		<u>conditions</u>	
	Domain 3	<u>Helping people to recover from episodes of ill-health or following injury</u> Emergency admissions for acute conditions that should not usually require hospital admission	✓
	Domain 4	<u>Ensuring people have a positive experience of care</u> Patient experience of \primary Care Patient experience of hospital care Friends and family Test <u>Improving children and young people's experience of healthcare</u> <u>Improving peoples experience of integrated care</u>	✓
	Domain 5	<u>Treating and caring for people in safe environment and protecting them from avoidable harm</u> Patient safety Incidents reported Reducing the incidence of avoidable harm	✓

3. Scope

3.1 Aims and objectives of service, including out of scope provision

The Patient Pathway Optimisation local service aims to support those patients whom a Primary Care Clinician would deem appropriate for a 'test' or procedure to enable them to diagnose a condition as well as funds clinical (and associated administrative) work to provide post-operative care within the primary care setting.

Provision of the following range of services during hours of provision as defined within the terms of the Practices medical services contract (*lists services*)

In addition, being mindful of the frequency with which activities undertaken by General Practice teams that are not formally commissioned / contracted are identified, that Practices commit where such a service is identified to continue provision in line with historic provision for a period of not less than 3 months from point of formal notification of issue.

The PPO is commissioned to fund existing and established (whether formally or by means of established practice) patient pathways within its scope. It is explicitly not commissioned to:

- Fund future patient pathways changes where such changes (whether formally agreed or otherwise implemented) impact on the volume of work required for any element of the PPO.
- Fund transfer of work currently provided by other system partners (such as Express

Diagnostics) when providers reach capacity.

- Fund additional capacity should current providers cease provision or where services are decommissioned.

3.2 Service description/care pathway

- **Phlebotomy:** Phlebotomy where need is identified within General Practice for purpose of diagnosis or condition management within primary care.
- **ECGs:** ECGs where deemed clinically appropriate for management within General Practice of long-term conditions or to address urgent cardiac need.
- **Ear wax removal:** Ear wax removal where clinical need is identified within General Practice, and for patients who have been sent to their GP by other providers where clinical need is confirmed by a clinical member of the primary care team.
 - Accumulation of wax in the ear canal is a very common issue for patients but usually can be managed conservatively – i.e. with the use of ear drops only. In many situations ear wax will not be causing symptoms and there is no necessity to remove the wax at all.

The PPO LES includes ear irrigation in certain circumstances. The decision whether or not to irrigate rests with the treating clinician.
 - Situations when ear irrigation as part of the PPO LES may be appropriate:
 - Hearing loss
 - When examination of the tympanic membrane is not possible due to ear wax and the patient has significant symptoms and where clinically appropriate.
 - Situations when ear irrigation is unlikely to be appropriate as part of the PPO LES
 - Wax accumulation but only mild symptoms – this may include mild hearing loss
 - Requests by external hearing test providers – they would be expected to manage the ear wax themselves with the recommendation of appropriate ear drops and review prior to signposting back to a GP.
- **Home Blood Pressure monitoring**
- **Doppler investigations of symptoms suggestive of peripheral vascular disease**
- **Removal of sutures and clips**
- **Dressing of pilonidal abscesses:** Provision by appropriately qualified members of the General Practice team, upon presentation, patient request, request from other healthcare professional, of a clinically appropriate dressing of pilonidal abscesses.
- **Management of post-operative need:**
 - The dressing and management of all types of surgical wounds including treatments for infected wounds and minor dehiscence as clinically and procedurally appropriate in a General practice setting
 - The dressing, management & closure of simple wounds, including those arising from pressure ulceration, and types of post-surgical wounds clinically appropriate for General Practice, including treating infected wounds and minor dehiscence as clinically and procedurally appropriate in a General practice setting.
 - Management of post-operative need where such falls within the competencies of General Practice, such as simple infection management, and has been historically managed within general practice.

- **Management of simple uncomplicated wounds:**
 - The dressing and management of simple wounds, including those arising from pressure ulceration and leg ulcers with signs of venous insufficiency (for the first two-weeks of non-healing after which a referral may be made to LLTS if felt to be clinically required)
 - Types of post-surgical wounds clinically appropriate for General Practice, including (not exhaustive) treating infected wounds, infected injection sites, and minor dehiscence (excluding those required for initial treatment of minor injury).
 - Management of simple uncomplicated wounds such as pressure ulceration
- **Closure of simple wounds using simple wound closure dressing or comparable technique:** Closure of simple wound using simple wound closure dressing or comparable technique where clinicians are satisfied adequate clinical skill and capability exists (excluding those required for initial treatment of minor injury).
- **Clinical coding:** clinical coding appropriate to management and care provided.
- **Recall or advanced booking:** Recall or advanced booking as deemed clinically appropriate to ensure provision of care within General Practice.

3.3 Training

Each practice must ensure that all staff involved in providing any aspect of care under this scheme have the necessary training and skills to do so.

3.4 Population covered

The registered population of the practice, along with any temporary residents requiring diagnostics and or post-operative care where local provision is more appropriate than care provided by their registered practice.

3.5 Any acceptance and exclusion criteria and thresholds

Exclusions include:

- Suturing of wounds or dressings arising from procedures undertaken by the practice such as minor operations or procedures arising from minor injuries
- Topical negative pressure therapy
- Larvae therapy
- Management of complex wounds and leg ulcers (venous, mixed and arterial), requiring compression bandaging or dressing of comparable complexity
- Housebound patients

3.6 Interdependence with other services/providers

Where diagnostic procedures have been carried out in primary care, results should be presented along with any referral, via DRSS choose and book. Those procedures deemed as diagnostic should not be repeated at the referral destination.

The practice will make all reasonable efforts to ensure appropriate liaison with relevant other providers within the patient's pathway.

Includes community nursing, MIU, Out of Hours provision, and acute units.

4. Applicable Service Standards

4.1 Applicable national standards (eg NICE)

The provider is required to adhere to all national standards as issued from time to time by any relevant Regulatory and Statutory bodies including guidance issued by appropriate competent bodies (eg Royal Colleges)

Phlebotomy – [WHO guidelines on drawing blood: best practices in phlebotomy](#)

4.2 Applicable local standards

None identified at this time.

Devon Formulary website - <https://devonformularyguidance.nhs.uk/>

5. Applicable quality requirements and CQUIN goals

5.1 Applicable Quality Requirements (See Schedule 4 Parts [A-D])

Not applicable

5.2 Applicable CQUIN goals (See Schedule 4 Part [E])

Not applicable

6. Key Service Outcomes

The service is designed to offer patients a high quality service that is both conveniently located and cost effective.

7. Information Requirements

No information is required from General Practice in terms of activity undertaken

8. Location of Provider Premises

The practice name and address as specified in the primary medical services contract.

9. Financial Requirements

For the financial year of 2026/27 the LES will be paid at £5.15 per weighted patient.

Payment is for the provision of **all** elements of the PPO, where clinically appropriate as set out in 3.2, to provide **full** coverage to **all** registered patients of the practice, including temporary residents requiring diagnostics and or post-operative care where local provision is more appropriate than care provided by their registered practice.

For 2026/27 Payments will be based on the following list size calculations:

- For the 1st two quarters of 2026/27 – based on the 31st March 2026 list size
- For the 2nd two quarters of 2026/27 – based on the 1st October 2026 list size

DDRB will be used as the start point for annual ‘inflationary’ review, with any deviation from DDRB uplift requiring clear rationale.

In the event of a pandemic or other ‘level 4 health’ event, commissioner and LMC will meet urgently to review this specification giving consideration to:

- a. suspension of some or all elements of it,
- b. redirection of patients to alternative provider(s),
- c. in the event of actioning either a) or b) determining appropriate income protection arrangements for GP provider

10. Notice Period

3 months’ notice will be required to terminate this contract by either party.